Question:

A baby is born after the 32nd gestational week by cesarean delivery. The mother suffered from gestational diabetes; however, she had no other pregnancy-related diseases and was otherwise healthy. The baby has a blood pressure of 100/58 mm Hg, heart rate of 104/min, and oxygen saturation of 88%. The child has tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis. The cyanosis is responding well to initial administration of oxygen. The nasogastric tube was positioned without problems. Which of the following is the most likely diagnosis?

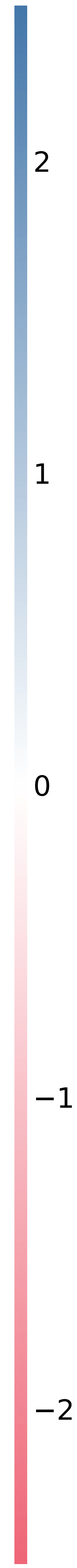
Answer Choices:

A. Neonatal respiratory distress syndrome (NRDS)

B. Tracheoesophageal fistula

C. Pneumonia

D. Congenital heart anomaly with right-to-left shunt



Reasoning + Answer:

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- The baby has signs of respiratory distress: tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis.
- The mother had gestational diabetes, but this is not directly related to the baby's symptoms.
- The baby's blood pressure and heart rate are within normal ranges for a newborn.
- The baby's oxygen saturation is low (88%), which is consistent with respiratory distress.
- The baby's ability to respond to oxygen administration suggests a reversible cause of the hypoxemia.
- NRDS is a common cause of respiratory distress in newborns, especially in premature babies. However, this baby is past the 32nd gestational week, which is a late preterm birth, and NRDS is more common in premature infants.
- Tracheoesophageal fistula would present with feeding difficulties and aspiration, but the nasogastric tube placement was uneventful.
- Pneumonia can cause respiratory distress, but it would typically present with additional signs such as fever and leukocytosis, which are not mentioned.
- Congenital heart anomalies with right-to-left shunts can cause cyanosis and respiratory distress, but they typically present with a heart murmur and are less likely to be reversible with oxygen alone.

Given the clinical presentation and the fact that the baby's condition improves with oxygen, the most likely diagnosis is Neonatal Respiratory Distress Syndrome (NRDS), although the baby is past term, it's still a possibility. However, the most fitting diagnosis considering the reversible response to oxygen is more likely to be a congenital heart anomaly with right-to-left shunt.

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<answer>D. Congenital heart anomaly with right-to-left shunt</answer>